

One Day, 301 People: What Your Support Made Possible at Abwoch

The official pre-launch medical outreach of the Acholi Women in Health Foundation, held under the theme "Rooted in Community, Rising in Health: The AWIHF Story", proved that integrated, partner-led care works and that the demand is real.



Community members gather before the health education talk at Abwoch Health Centre III.

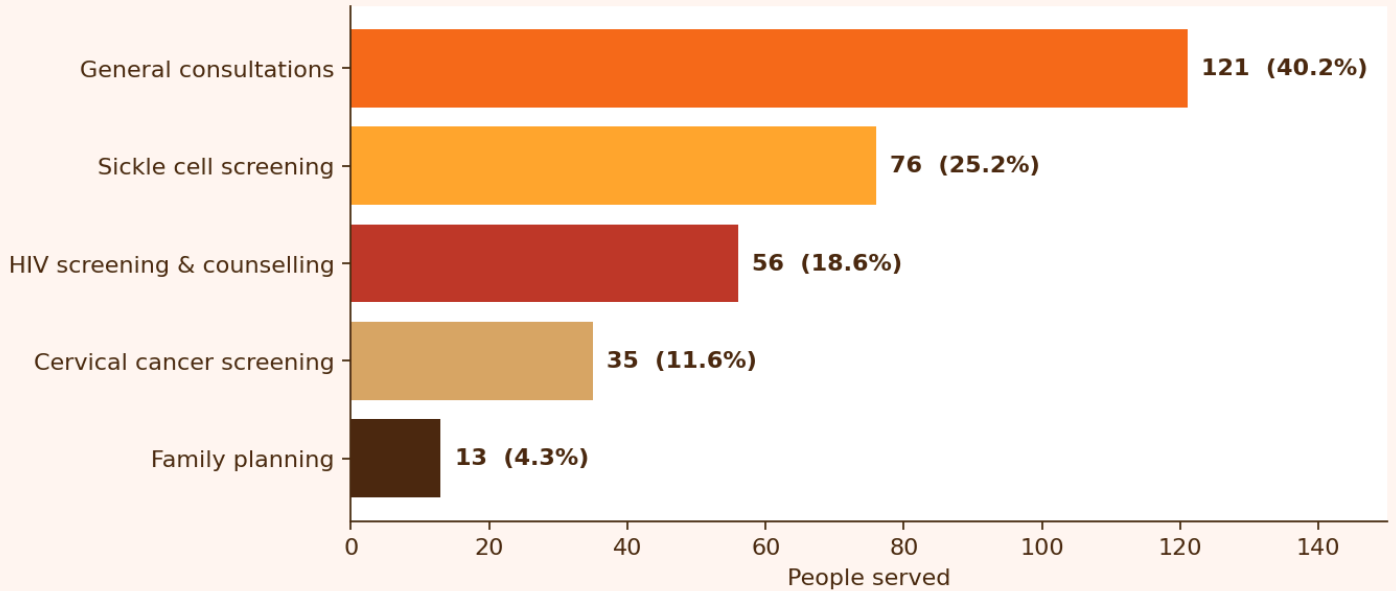
On 3 July 2026, the Acholi Women in Health Foundation (AWIHF) held its official pre-launch medical outreach at Abwoch Health Centre III in Tochi County, Omoro District, under the theme **"Rooted in Community, Rising in Health: The AWIHF Story."** The camp was part of the Foundation's pre-launch programming, designed to demonstrate our service model and reach underserved communities before our official public launch. It brought five essential health services to the community, free of charge. By the end of a single day, we had reached **301 community members**. The camp exceeded its targets, and it showed what a coordinated, multi-partner model can deliver in a community facing real barriers of distance, cost and awareness.

This is proof that your investment works. Here is what one day delivered, what it taught us, and where your next contribution can change the most lives.

Impact at a glance

- **301** people reached in one day, across five services
- **121** general consultations, with **71** ending in a documented diagnosis and treatment
- **76** people screened for sickle cell, and **23** found to carry the trait or the disease
- **56** people reached with HIV screening and counselling, with prevention and care linkage for those who needed it
- **35** women screened for cervical cancer, with **1** referred for further management
- **13** women supported with modern family planning
- **5** partners working side by side

Service uptake by category (total = 301)

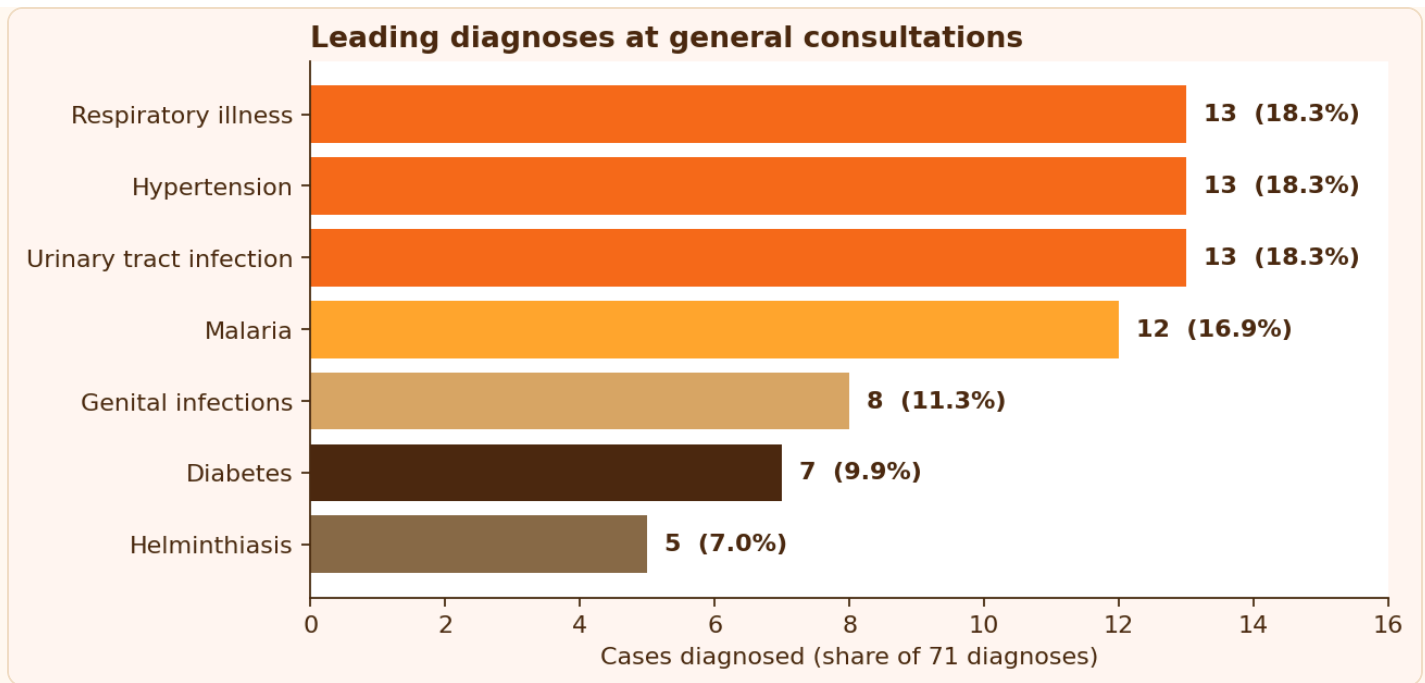


Service	People served	Share of camp total
General medical consultations	121	40.2%
Sickle cell screening	76	25.2%
HIV screening and counselling	56	18.6%
Cervical cancer screening	35	11.6%
Family planning	13	4.3%
Total	301	100%

Where your support went

Treating everyday illness that often goes untreated

General consultations were the busiest service, with 121 people seen (40.2% of the camp). Women made up 71.1% of them. Seventy-one consultations ended in a documented diagnosis and treatment, which shows how much unmet need for basic curative care there is in the community.



Respiratory illness, hypertension and urinary tract infections each accounted for 13 diagnoses (18.3%), followed by malaria (12, 16.9%), genital infections (8, 11.3%), diabetes (7, 9.9%) and helminthiasis (5, 7.0%). Hypertension and diabetes are conditions that need ongoing follow-up, which is why referral pathways are a priority for the next phase.

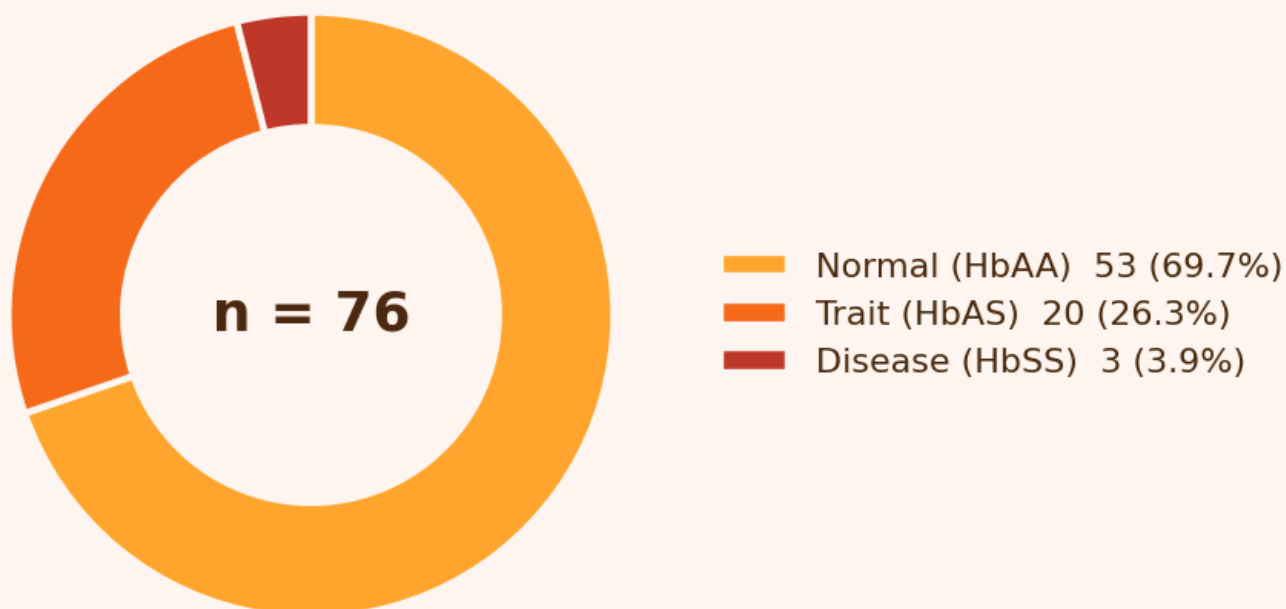


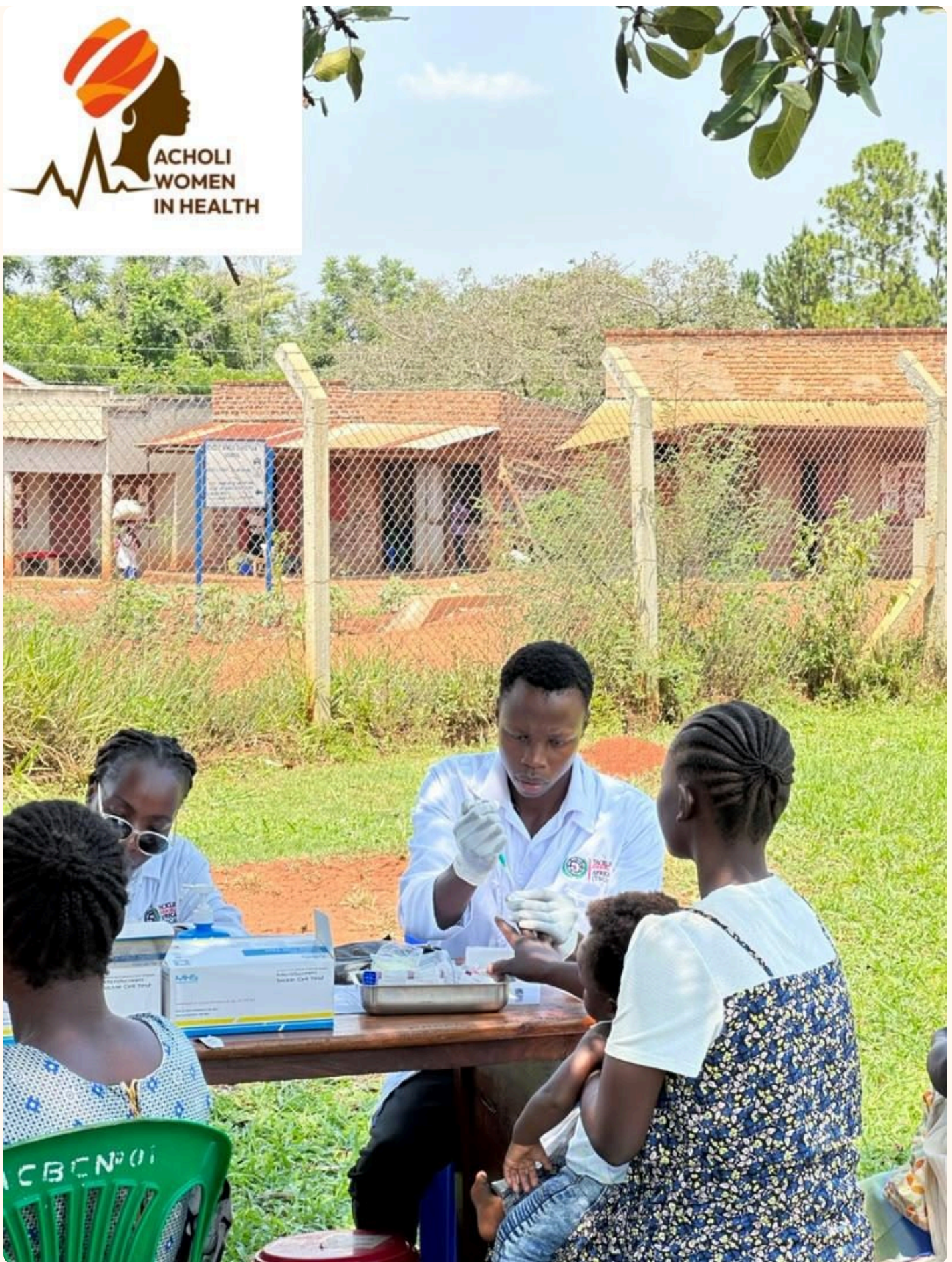
AWIHF's Co-Founder and Chief Operations Officer carrying out a general examination in the consultation room.

Knowing your sickle cell status: 76 people screened

Delivered with Tackle Sickle Cell Africa, the screening found 53 people (69.7%) with normal haemoglobin (HbAA), 20 (26.3%) with sickle cell trait (HbAS) and 3 (3.9%) with sickle cell disease (HbSS). About 30% of those screened carried the trait or the disease. Knowing their status lets these 23 people make informed family planning decisions, which helps reduce future sickle cell births.

Sickle cell screening results

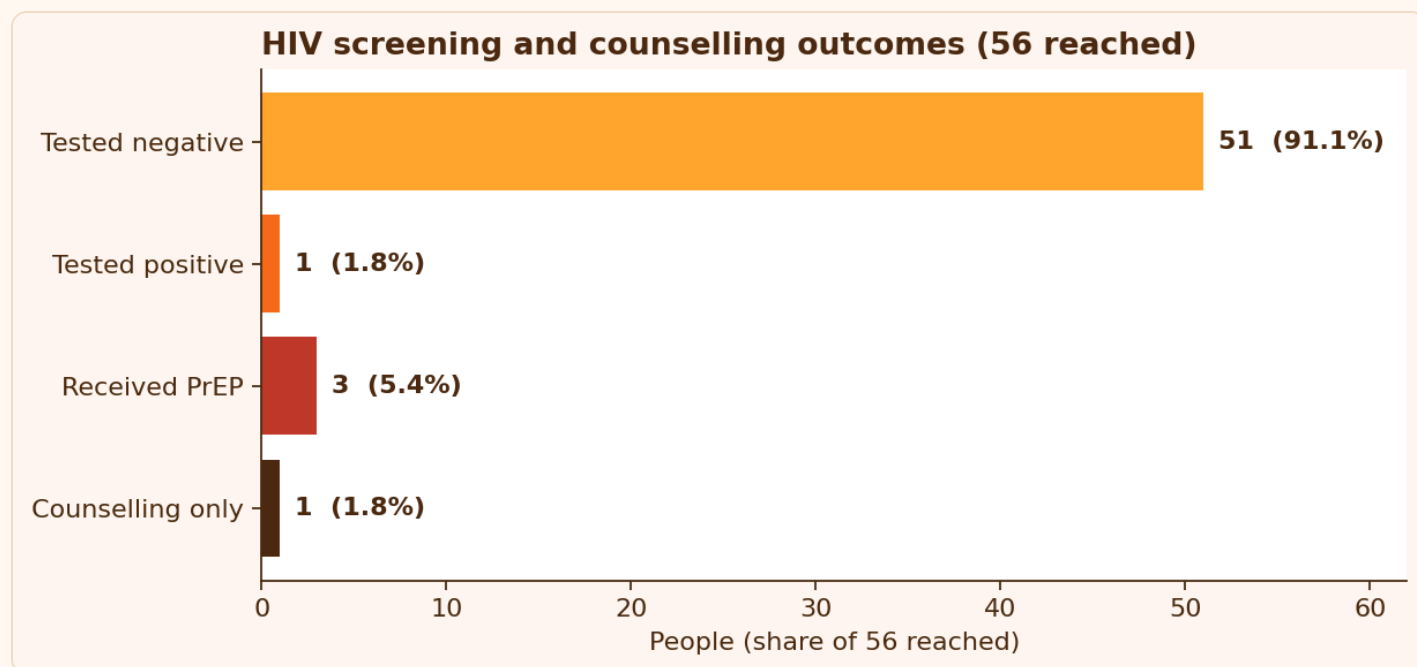




Tackle Sick Cell Africa staff counselling and screening clients.

HIV testing, prevention and care: 56 people reached

With The AIDS Support Organisation (TASO) Gulu, we reached 56 community members. Of the 52 people tested, 51 (98.1%) were negative and one (1.9%) tested positive and was linked to follow-up counselling and care. Three people were started on pre-exposure prophylaxis (PrEP) and one received counselling only.

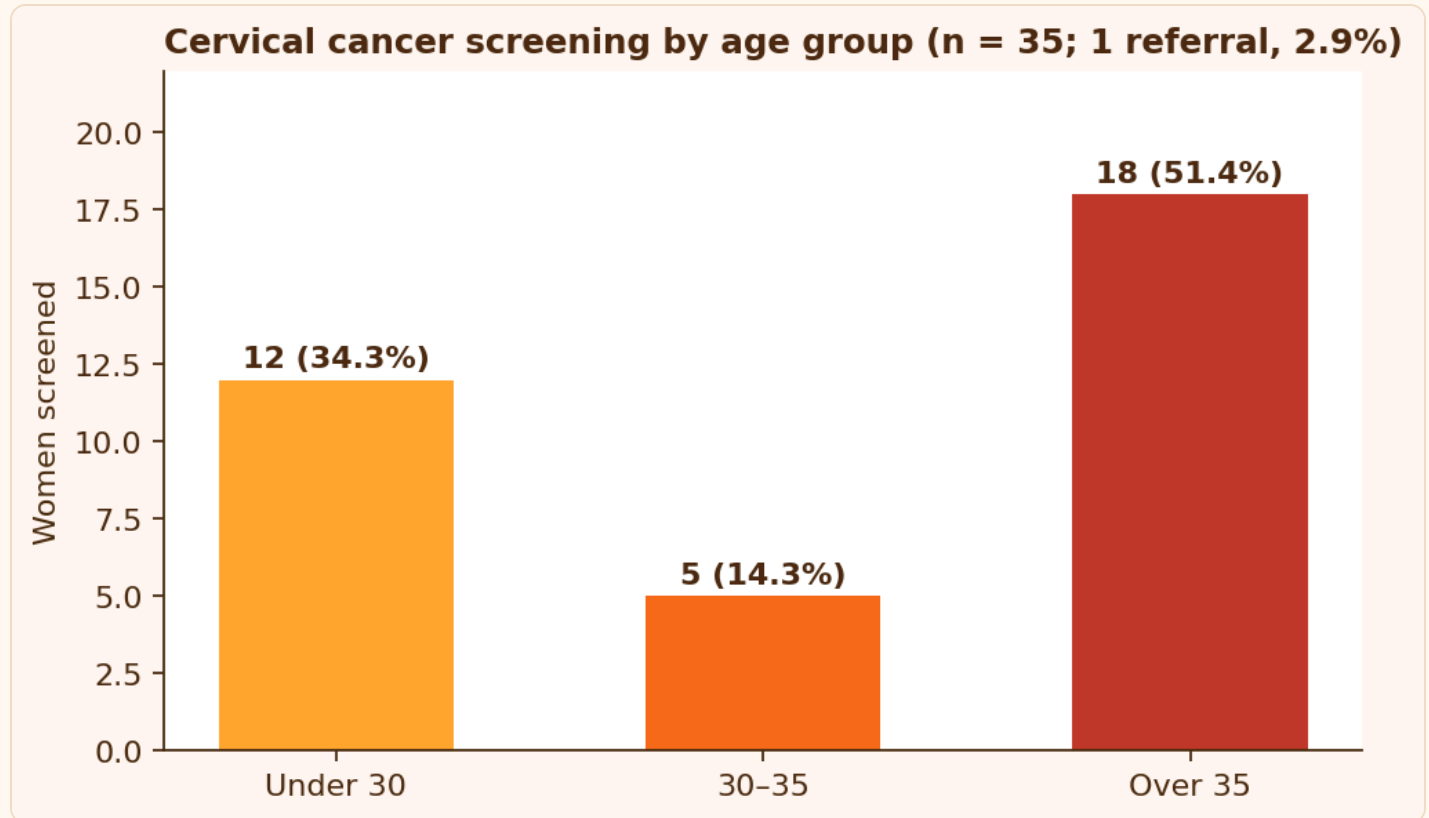




A TASO staff member educating participants on HIV self-testing using OraQuick.

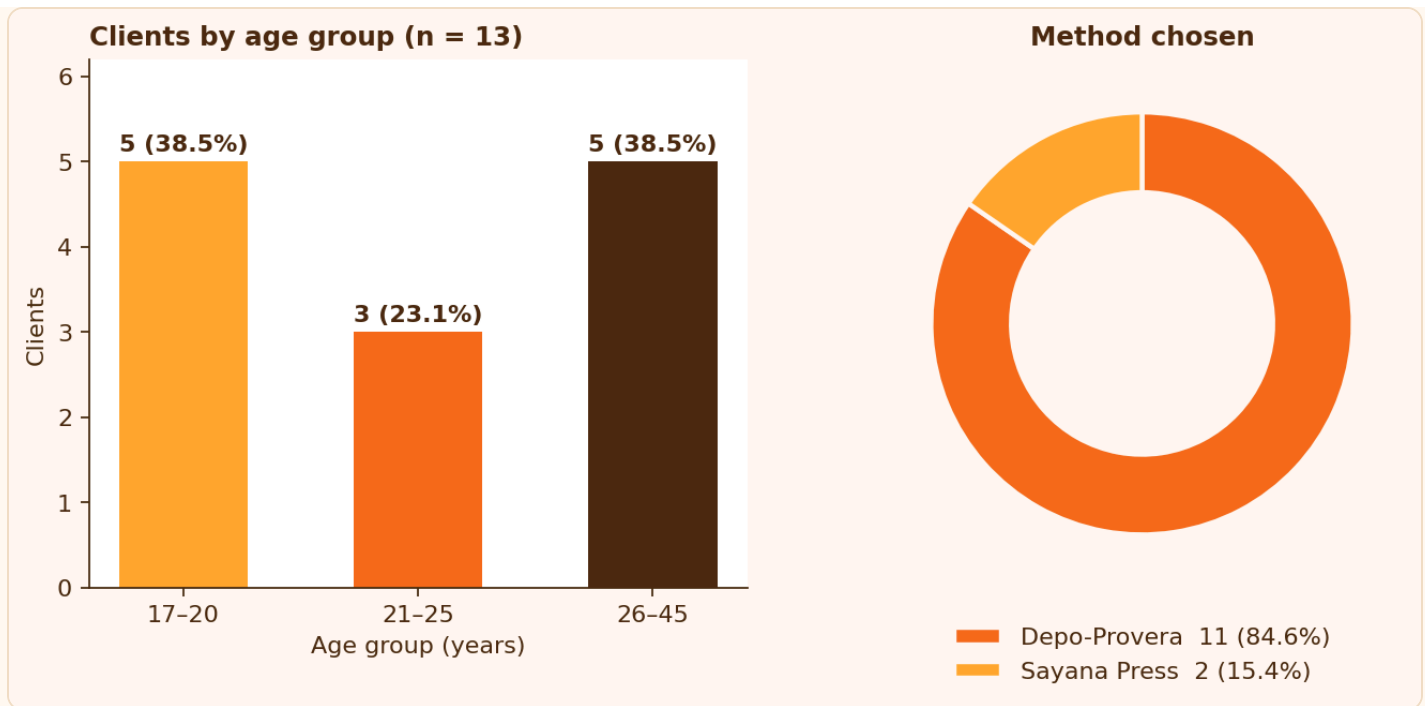
Cervical cancer screening: 35 women

With Reproductive Health Uganda (RHU), 35 women were screened: 12 under 30 (34.3%), 5 aged 30–35 (14.3%) and 18 over 35 (51.4%). One woman (2.9%) was referred for further assessment and linked to management. Nearly half of those screened (48.6%) were 35 or younger, in line with the WHO goal of screening 70% of women by age 35.



Family planning: 13 women

Clients ranged in age from 17 to 45. Five were aged 17–20 (38.5%), three were 21–25 (23.1%) and five were 26–45 (38.5%). Eleven chose Depo-Provera (84.6%) and two chose Sayana Press (15.4%). The strong uptake among younger women shows the need for more access and awareness in a region heavily affected by teenage pregnancy.



Our partners: thank you

No single organisation could have delivered this day alone. Task-sharing and cost-sharing between our partners allowed broader coverage than any one of us could have provided. We are deeply grateful to:

- **Tackle Sickle Cell Africa (TSCA)**, for providing sickle cell counselling and screening.
- **The AIDS Support Organisation (TASO), Gulu Centre**, for providing HIV counselling and screening.
- **Reproductive Health Uganda (RHU), Gulu Branch**, for providing cervical cancer screening and awareness, and family planning services.
- **Abwoch Health Centre III and Omoro District Local Government**, for hosting the outreach and providing the infrastructure and utilities that made it possible.
- **Hashtag Gulu**, for standing with us and helping us reach the community.
- **Our staff and volunteers**, who gave their time and skill to organise and deliver the day.



AWIHF's Founder and CEO speaking with an RHU staff member during the outreach.

What we learned, and why it matters for the next phase

The strongest sign of success was also our biggest challenge: more people came than we planned for.

- **Demand outstripped capacity.** Turnout was higher than expected, and there were not enough clinicians, which led to long waits at consultation, laboratory and dispensary points.
- **Supplies ran short.** Some participants missed out on curative and preventive drugs.
- **No formal triage or patient flow system.** Acutely ill patients mixed with those coming for preventive screening, and there was congestion at a single registration point.
- **One day limits depth.** A single-day model restricted the time available for counselling at every service point.

We see these as a clear guide for where investment will go furthest.

Where your continued support will make the difference

The outreach validated a coordinated, single-day, multi-service model that can be replicated across Omoro District and the wider Acholi sub-region. To scale it well, AWIHF's priorities are to:

1. **Fund more clinicians** at consultation points, to cut waiting times and reduce the load on a small team.
2. **Procure and quantify supplies early**, so no participant leaves without the drugs they need.
3. **Build structured referral and follow-up pathways** for people found with sickle cell trait or disease, hypertension, diabetes and HIV.
4. **Expand cervical cancer screening and family planning outreach**, where uptake was lower than for consultations and sickle cell screening.
5. **Train more staff and volunteers** in documentation tools, so we can measure and report impact even more accurately.
6. **Replicate the model** at additional health centres across the Acholi sub-region.

Partner with us

If you are a donor, funder or partner who would like to help take this model further, we would be glad to talk.

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To every donor, sponsor and partner, and to the people of Abwoch who trusted us with their health: thank you.

Healthier women. Healthier families. Stronger communities. Together for Lasting Change.